THE MATERNITY METHOD

NCLEX CHEAT SHEET

Preeclampsia

Maternal & Newborn › Hypertensive Disorders

The Big Picture

Preeclampsia = **new high BP after 20 weeks** plus signs that **organs are stressed** (protein in urine, low platelets, liver or kidney changes, headache, vision changes). The placenta causes **blood vessel spasm** → less blood to mom's organs **and** to baby. **The only cure is delivering the placenta**, and it can still happen **up to 6 weeks postpartum**.

KNOW THE NUMBERS

≥ 140 / 90	≥ 160 / 110	< 100,000	4–7 mEq/L	RR < 12
Twice, **4 hrs apart**, after 20 wks	**Severe**: treat within 30–60 min	**Platelets**: severe feature	Therapeutic **magnesium**	**Stop** magnesium

The Spectrum

Condition	What it means
Gestational hypertension	BP ≥ 140/90 after 20 wks, **no** protein or organ signs
Preeclampsia	BP ≥ 140/90 after 20 wks **+ proteinuria** (≥ 300 mg/24 hrs, protein/creatinine ≥ 0.3, or dipstick 2+) **or** severe features
With severe features	BP **≥ 160/110**, platelets < 100,000, **liver enzymes 2× normal**, **RUQ/epigastric pain**, creatinine > 1.1, **pulmonary edema**, new **headache**, **vision changes**
Eclampsia	**Seizure** (can happen before, during, or after birth)
HELLP syndrome	**H**emolysis · **E**levated **L**iver enzymes · **L**ow **P**latelets. May have **upper belly pain** and nausea; BP may only be mildly high

Who's at Risk

- **First** pregnancy; **prior** preeclampsia
- **Chronic HTN**, diabetes, **kidney** disease, lupus
- **Twins** or more
- **Obesity**, age ≥ 35, IVF

High-risk? **Low-dose aspirin** (81 mg) starting at 12–28 wks (best before 16) **lowers risk**.

Spot It

- **Headache** that won't go away
- **Blurry vision**, spots, light sensitivity
- **RUQ or epigastric pain** (liver)
- **Sudden swelling** of face/hands, fast weight gain
- **Brisk reflexes** (3+, 4+), **clonus**
- Low urine output, **crackles**

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Nursing Actions

Goal: **prevent seizures** + **control BP** + **watch baby**

STEP 1
Assess

- **BP**: seated, right-size cuff, arm at heart level
- **DTRs + clonus**, LOC, headache, vision, lungs
- **I&O**, daily weight, urine protein
- Labs: platelets, **liver enzymes**, creatinine; **NST**/FHR

STEP 2
Treat

- **Magnesium sulfate** to **prevent seizures** (often continued about 24 hrs after birth)
- **BP meds** for ≥ 160/110: labetalol, hydralazine, or nifedipine
- **Betamethasone** if < 34 wks
- **Delivery** is the cure; timing depends on severity

STEP 3
Protect

- **Seizure precautions**: padded rails, **suction + O_2 at bedside**, call light
- **Quiet, dim** room; limit visitors
- Watch for **mag toxicity** (below)

Magnesium Safety

- Check **RR, DTRs, LOC, urine output** often
- **Toxic: RR < 12, absent DTRs, urine < 30 mL/hr**, slurred speech
- **Stop** the infusion → **calcium gluconate** IV
- Expected: **flushing**, warmth, drowsiness

During a Seizure

- **Call for help**; stay with her; note the **time**
- **Side-lying**, protect head; **don't restrain**
- **Nothing in the mouth**
- After: **suction**, **O_2**, check FHR, magnesium as ordered

MEMORY HACK · DTR GRADING

Reflexes: 2+ is normal

0	1+	2+	3+	4+
absent: mag toxicity?	low / sluggish	**normal**	**brisk**	very brisk with **clonus**

RED FLAGS · STOP AND CALL THE PROVIDER

BP ≥ 160/110 · **Severe headache** or vision changes · **RUQ/epigastric pain** · **Seizure** · RR **< 12**, absent DTRs on mag · **Shortness of breath** / crackles · Postpartum **headache + high BP**

MUST-KNOW FOR THE NCLEX

- **Cure = delivery of the placenta**, but it can still happen up to 6 weeks postpartum.
- **Magnesium prevents seizures.** It is not mainly a BP medicine.
- **Mag toxicity:** RR < 12, absent DTRs, urine < 30 mL/hr → stop + **calcium gluconate**.
- **Severe BP ≥ 160/110** → treat within 30–60 min.
- **Epigastric/RUQ pain** = liver involvement → think **HELLP**.

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QUICK CHECK · TEST YOURSELF

1. The main purpose of magnesium sulfate in preeclampsia is to prevent _____.
2. On magnesium, an RR of 10 and absent DTRs → stop the drip and give _____.
3. HELLP stands for _____.
4. Severe-range BP is ≥ ____ / ____.
5. Which low-dose medicine can lower preeclampsia risk in high-risk patients?

Answers: 1. seizures · 2. calcium gluconate · 3. Hemolysis, Elevated Liver enzymes, Low Platelets · 4. 160 / 110 · 5. aspirin (81 mg)

For study use only. Always follow facility protocols, current guidelines, and provider orders.